GSVFD First Responder EMS Instructor Guide

Oct 12, 2026

Course overview

This course teaches GSVFD members to start patient care under the Wise County First Responder protocols and hand the patient off to Wise County EMS. It runs eight modules (about 20 classroom hours) and a scenario lab, about 47 hours in all, and each module ends at the matching EMS sign-offs (E.1–E.26) in the Firefighter Taskbook.

The course is protocol and skills training, not a certification course. Members still earn their EMS certification (ECA, EMR or EMT) through a course approved by the Texas Department of State Health Services. The level GSVFD requires is set by the department under NFPA 1010 Chapter 10.

Who does what under the protocols

Level	What the protocols allow
All first responder (FRO) personnel	Every protocol's main steps: assessment, airway, oxygen, bleeding control, CPR-AED, splinting, positioning, cooling and warming, childbirth
EMT-B and above	Also: assist with the patient's own epinephrine auto-injector, rescue inhaler or nebulizer, and nitroglycerin; give aspirin; give oral glucose
Highest-certified member on scene	Leads patient care until Wise County EMS arrives, then gives the handoff report

Modules

Module	Title	Protocols covered	Taskbook	Class hrs	Skills hrs
E1	First responder role and general orders	II, IV, V, VI	E.1–E.3, E.25	2	1
E2	Patient assessment and vital signs	II, Appendix 1, Appendix 3	E.4–E.7	3	3
E3	Airway, oxygen and CPR-AED	10	E.8–E.10	2	4
E4	Medical emergencies	1, 4, 7, 11, 12, 21	E.18–E.20	3	2
E5	Environmental, poisoning and behavioral	9, 13, 17, 19, 20, 24	E.21–E.23	3	1
E6	Bleeding, shock, soft tissue and burns	2, 3, 5, 6, 14, 23, Appendix 2	E.11–E.15	3	3
E7	Fractures, spine and multiple trauma	15, 16, 18	E.16–E.17	2	3
E8	Childbirth and sensitive patients	8, 22, 25, Appendix 3	E.23–E.24	2	2
Lab	EMS scenarios	All	E.26	0	8
	Total			**20**	**27**

Hours are planning estimates, not a state or NFPA minimum. Teach E1–E3 first; a new member can then start CPR, control bleeding and give a handoff report on a call. Run the scenario lab after E8, or one or two scenarios at the end of each module.

Instructor requirements, safety and protocol notes

Teach and test every skill exactly as the Wise County First Responder protocols are written; they are the medical direction GSVFD operates under. Where a protocol differs from current national guidance, raise it with the Wise County EMS medical director (list below) rather than teaching around it.

Who may teach and sign

Role	Minimum qualification	Signs in the taskbook
Lead instructor	Current Texas DSHS EMT or paramedic; DSHS EMS instructor preferred	K, D and P for any EMS task

Role	Minimum qualification	Signs in the taskbook
Skills evaluator	Certified at or above the level of the skill (EMT-B items: EMT or higher)	P
Scenario operator	Any certified member who knows the scenario	Runs the scenario; the evaluator signs

Every session

- Gloves and eye protection for every hands-on station, as on a real call.
- Clean manikin faces and airway heads between users with the manufacturer's wipes; use one-way valves or BVMs, never mouth-to-mouth on a manikin.
- No invasive skills on classmates. Practice positioning, splinting and assessment on volunteers only with their consent; use moulage, not real cuts.
- Outdoor scenario labs follow the department heat and rehab rules.
- Section III (Equipment List) is listed in the protocol contents but is missing from the copy used for this guide. Get the current list from Wise County EMS and check each apparatus against it before the course.

Protocol items to confirm with the medical director

Topic	Wise County protocol	Current national guidance
Severe extremity bleeding (3)	Direct pressure and elevation, then a BP cuff or tourniquet if bleeding persists	Stop the Bleed teaches an early tourniquet for life-threatening limb bleeding and no longer uses elevation
Rescue breathing rate (16, 18)	BVM at 24 breaths per minute	AHA: about 1 breath every 6 seconds (10 per minute) for an adult with a pulse
Oxygen (many protocols)	Non-rebreather at 12–15 L/min for most patients, including chest pain and stroke	AHA no longer recommends routine oxygen for chest pain or stroke patients whose oxygen saturation is normal
Spinal injury (16)	Immobilize to a long spine board	Joint EMS and trauma position statements favor spinal motion restriction, with long boards mainly for extrication

Topic	Wise County protocol	Current national guidance
Sucking chest wound (6)	Occlusive dressing taped on three sides	Many trauma courses now use a vented chest seal, or a full seal that is burped if breathing worsens
Newborn care (8)	Suction mouth then nose on every delivery; clamp cord promptly	Neonatal resuscitation guidance suctions only an obstructed airway and delays cord clamping 30–60 seconds in a vigorous baby
Heat stroke (17)	Ice packs to neck, armpits and groin, or a wet sheet	Cold-water immersion is the fastest cooling where available; cool on scene before transport
Frostbite rewarming (9)	Water at 100–105°F	Wilderness medicine guidance uses about 98.6–102°F (37–39°C)
Refusals (IV)	Pregnant patients of any age and minors not living with an adult family member may refuse	Who may consent or refuse is set by Texas law; confirm the wording with the medical director and legal counsel

Until the medical director changes a protocol, trainees are tested on the protocol as written. Use this table for the instructor's conversation with Wise County EMS, not for the trainees.

Module E1 — First responder role and general orders

Protocols: II General Orders, IV Cancellation, V Air Evacuation, VI Accompanying Patients · **Taskbook:** E.1–E.3, E.25 · **Time:** 2 hr class, 1 hr skills

Objectives. The trainee can explain who leads care on scene and when care passes to Wise County EMS, list the 11 general orders in order, give a handoff report, handle a refusal and cancel the ambulance only when the protocol allows, name the air evacuation criteria, follow the ride-along rules, and complete the First Responder report and refusal forms.

Materials. Wise County First Responder protocols, blank First Responder report and refusal forms, Module E1 slides.

Lecture outline

1. The highest-certified GSVFD member leads patient care until Wise County EMS arrives.
2. Care passes to Wise County EMS when its medic makes patient contact, after a handoff report of findings and treatments.
3. The 11 general orders: scene safety; responsiveness; ABCs; severe bleeding; level of consciousness; reassure; brief history; vitals and exam (check for a medical alert tag); medications and allergies; treat by protocol in priority order; complete the report form.
4. Cancelling the responding unit: only for the patients and decision-makers the protocol lists (adults who are conscious and fully oriented, parents and guardians for a minor, holders of a healthcare power of attorney unless the patient is alert and wants care, and the other listed groups). Have the patient sign the First Responder refusal form.
5. Air evacuation criteria: terrain or access, level of care not available locally (major burns, neurosurgery), no ground units available, penetrating trauma to head, neck or chest, obvious spinal cord or closed head injury, amputation that may be reattached, priapism.
6. Riding in the ambulance: only when needed and the medic approves; work under the medic's direct supervision; wear a seat belt; follow HIPAA.
7. Documentation: why the report form matters, what goes in it, and keeping patient information private.

Skills stations

- **A. Handoff:** each trainee gives a 30-second report to an "arriving medic" from a scenario card.
- **B. Refusal:** role-play a refusal, check the patient is alert and oriented, explain risks, complete the form.

Evaluation and common errors. Watch for handoffs that ramble or leave out treatments given, cancelling EMS for a patient who is not fully oriented, and incomplete refusal forms.

Instructor notes. Use the handoff format in the Scenario evaluation section. Confirm with Wise County EMS how GSVFD should request air evacuation (directly, or through the responding medic).

Module E2 — Patient assessment and vital signs

Protocols: II General Orders, Appendix 1 (Glasgow Coma Scale), Appendix 3 (Patient Assessment Chart) · **Taskbook:** E.4–E.7 · **Time:** 3 hr class, 3 hr skills

Objectives. The trainee can size up a scene, form a general impression, run an initial assessment (responsiveness, ABCs, severe bleeding, level of consciousness), take a SAMPLE history and an OPQRST pain history, do a DCAP-BTLS physical exam, take a full set of baseline vital signs, score the Glasgow Coma Scale, and know how often to repeat vitals.

Materials. BP cuffs and stethoscopes, pulse oximeters, penlights, watches with second hands, the patient assessment chart, moulage, volunteer patients.

Lecture outline

1. Scene size-up: hazards, gloves and eye protection, number of patients, mechanism of injury or nature of illness, more help needed.
2. Initial assessment: general impression, responsiveness, airway, breathing, circulation, severe bleeding. Fix life threats as you find them.
3. Level of consciousness and the Glasgow Coma Scale (eyes 4, verbal 5, motor 6; best 15, worst 3; sternal rub as the pain stimulus).
4. SAMPLE: signs and symptoms, allergies, medications, pertinent past history, last oral intake, events leading up.
5. OPQRST for pain: onset, provocation, quality, radiation, severity (0–10), time.
6. DCAP-BTLS head to toe: deformities, contusions, abrasions, punctures, burns, tenderness, lacerations, swelling. Check for a medical alert tag.
7. Baseline vitals: breathing, pulse, skin, pupils, blood pressure, pulse oximetry.
8. How often to repeat: every 3–5 minutes for unstable patients and every 10 minutes for stable ones, or as each protocol states.

Glasgow Coma Scale (Appendix 1)

Score	Eyes	Verbal	Motor
6			Obeys commands
5		Oriented	Localizes pain
4	Open on their own	Confused	Withdraws from pain
3	Open to voice	Inappropriate words	Flexion to pain
2	Open to pain	Incomprehensible sounds	Extension to pain
1	None	None	None

Add the three columns: 15 is the best score and 3 the worst. Use a brisk sternal rub as the pain stimulus, and note any existing condition that prevents a full score.

Typical adult resting vitals (for teaching what "normal" looks like): respirations 12–20 per minute, pulse 60–100 per minute, systolic blood pressure about 90–140 mmHg, oxygen saturation 95% or higher, skin warm, pink and dry.

Skills stations

- **A. Vitals:** each trainee takes three full sets on volunteers, then on a timed lap.
- **B. Head-to-toe:** DCAP-BTLS on a moulaged volunteer while calling findings aloud.
- **C. History:** SAMPLE and OPQRST from an instructor playing a patient.
- **D. GCS:** score five video or role-played patients.

Evaluation and common errors. Watch for skipping scene safety, counting pulse or breaths for too short a time, not repeating vitals, and leading the patient in OPQRST ("Is it crushing?").

Instructor notes. Children's normal vitals differ by age; teach them from the department's EMS textbook.

Module E3 — Airway, oxygen and CPR-AED

Protocols: 10 CPR-AED, plus the airway steps in every protocol · **Taskbook:** E.8–E.10 · **Time:** 2 hr class, 4 hr skills

Objectives. The trainee can open an airway with a head-tilt chin-lift or a modified jaw thrust, suction, place a patient in the recovery position, give oxygen by non-rebreather mask, ventilate with a bag-valve mask (BVM) and oxygen, and run CPR with an AED on adults and children, including pediatric pads.

Materials. Adult, child and infant CPR manikins with feedback if available, AED trainer with adult and pediatric pads, BVMs (adult, child, infant), oxygen cylinder and regulator, non-rebreather masks, portable suction, Module E3 slides.

Lecture outline

1. Opening the airway: head-tilt chin-lift for medical patients; modified jaw thrust whenever spinal injury is possible (protocols 16 and 18).
2. Keeping it open: suction, and the recovery (side) position for unconscious patients who are breathing.

3. Oxygen: cylinder safety, regulator, non-rebreather mask at 12–15 L/min (the rate most protocols use); some protocols use 10–12 L/min.
4. BVM: mask seal with two rescuers, oxygen attached, watch for chest rise, avoid over-ventilation.
5. CPR-AED protocol: CPR for patients who are unresponsive, pulseless and not breathing; never for a patient who is conscious, has a pulse or is breathing.
6. The sequence: start CPR, attach the AED as soon as it arrives, stop CPR and press Analyze, follow the AED prompts, check for a pulse after analysis or signs of life.
7. Pediatric pads for a child under 8 years or under 90 pounds.
8. Continue until Wise County EMS takes over or rescuers are too fatigued; rotate compressors.

Skills stations

- **A. Airway:** manual maneuvers, suction, recovery position.
- **B. Oxygen and BVM:** set up the cylinder and NRB; two-rescuer BVM with oxygen on a manikin.
- **C. CPR-AED:** adult two-rescuer CPR with the AED trainer through two analysis cycles; then child with pediatric pads.

Evaluation and common errors. Watch for long pauses in compressions, shallow or slow compressions, ventilating too fast, forgetting to stand clear before a shock, and adult pads on a small child.

Instructor notes. The protocol relies on the AED's prompts. Teach compression depth, rate and ratios from the department's current CPR course (AHA or Red Cross) so they match each member's CPR card.

Module E4 — Medical emergencies

Protocols: 1 Allergic Reaction, 4 Breathing Difficulties, 7 Chest Pain, 11 CVA-TIA (Stroke), 12 Diabetic Emergencies, 21 Seizures · **Taskbook:** E.18–E.20 · **Time:** 3 hr class, 2 hr skills

Objectives. The trainee can recognize each condition, carry out the protocol's steps in order, know which medication assists are EMT-B and above only, and reassess at the protocol's vital sign interval.

Materials. Oxygen and NRB masks, BVM, trainer epinephrine auto-injector and inhaler, empty nitroglycerin bottle, aspirin and oral glucose training packages, orange juice and sugar packets for demonstration, suction, Module E4 slides.

Protocol summary

Protocol	All first responders	EMT-B and above also	Vitals
1 Allergic reaction	Open airway; BVM with oxygen if needed; NRB 12–15 L/min; treat for shock	Assist with the patient's own epinephrine auto-injector	Every 3–5 min
4 Breathing difficulty	Patent airway; BVM if needed; sit the patient up; NRB 12–15 L/min; detailed history and exam	Assist with the patient's own inhaler or nebulizer; check the expiration date	Every 3–5 min
7 Chest pain	Treat as a heart attack; loosen clothing; position of comfort; keep calm and still; NRB 12–15 L/min; OPQRST	Assist with the patient's nitroglycerin unless systolic BP is below 100 (no more than 3 doses); give 2 chewable 81 mg aspirin	Every 3–5 min
11 Stroke or TIA	Be ready to suction; NRB 12–15 L/min; neuro exam; semi-Fowler's; unconscious: BVM if needed and lateral recumbent	—	Every 3–5 min
12 Diabetic, conscious	If alert, has a gag reflex and can drink unaided: sugar drink (orange juice with 2–3 tablespoons of sugar)	Oral glucose if alert and able to swallow	Every 5–10 min
12 Diabetic, unconscious	Contact EMS; suction; recovery position; NRB 10–12 L/min; nothing by mouth	—	—
21 Seizures	Move to the floor; protect from harm; never restrain or put anything in the mouth; after the seizure, assess, suction or position, NRB 12–15 L/min	—	Every 5 min

Lecture outline

1. Anaphylaxis: hives, swelling, wheezing, low blood pressure; it can worsen in minutes. Look for the patient's auto-injector.
2. Breathing difficulty: asthma, COPD, heart failure; tripod position, accessory muscles, speaking in short phrases.
3. Chest pain: 15–20% of heart attack patients have no pain or atypical pain (back, abdomen), especially older adults, women and diabetics. Ask about erectile dysfunction drugs before nitroglycerin, as your EMT course teaches.
4. Stroke and TIA: facial droop, arm drift, slurred speech; a TIA patient whose symptoms resolve is still at high risk. Note the time the patient was last known well for the handoff.
5. Diabetic emergencies: low blood sugar comes on fast (confused, sweaty, combative); nothing by mouth unless the patient is alert with a gag reflex.
6. Seizures: protect, time the seizure, recovery position after; ask about epilepsy history and medications.

Skills stations

- **A. Medication assists (EMT-B and above):** the five rights with trainer auto-injector, inhaler and nitroglycerin; check the patient's name and expiration date.
- **B. Positioning:** semi-Fowler's, position of comfort, recovery position.
- **C. Mini-scenarios:** run scenarios 1–4 from this guide.

Evaluation and common errors. Watch for giving nitroglycerin without checking blood pressure, oral sugar to a patient who can't protect the airway, and restraining a seizing patient.

Module E5 — Environmental, poisoning and behavioral emergencies

Protocols: 9 Cold Emergencies, 13 Emotionally Disturbed Patients, 17 Heat Emergencies, 19 Poisoning, 20 Radiation Exposure, 24 Snakebite · **Taskbook:** E.21–E.23 · **Time:** 3 hr class, 1 hr skills

Objectives. The trainee can tell heat exhaustion from heat stroke and treat each, manage frostbite and hypothermia (including the 30-minute EMS ETA rule), handle ingested, inhaled and injected poisonings, give snakebite care without a tourniquet or ice, stay safe at a radiation incident, and approach an emotionally disturbed patient safely with law enforcement.

Materials. Cold packs, sheets and spray bottle, blankets, hot packs, splint materials, a mock poison container, Module E5 slides.

Heat and cold at a glance

Condition	Signs	First responder care (protocol)
Heat exhaustion	Cool, clammy skin	Move to a cool place; lay supine; vitals every 10 min
Heat stroke	Core temperature over 104°F, hot dry skin, altered mental status	Contact EMS; cool place; ice packs to head, neck, armpits and groin (or a wet sheet); stop if shivering; NRB 10–12 L/min; vitals every 5 min
Frostbite	1st: redness and swelling; 2nd: clear blisters; 3rd: blood-filled blisters	Remove clothing from the area; rewarm in 100–105°F water until numbness resolves; dry sterile dressing; elevate and immobilize
Hypothermia	Cold, shivering or not, altered	Contact EMS; remove wet clothing; dry blankets; warm room; if EMS ETA is under 30 min do not rewarm; over 30 min use a rewarming blanket or hot packs to head, neck, armpits and groin

Lecture outline

1. Heat illness on hay, livestock and roadside calls; heat stroke is a life threat, so cool first.
2. Cold: rough handling can trigger cardiac arrest in severe hypothermia; move gently.
3. Poisoning by route. Ingested: what, when and how much; relay to EMS early; bring the container and any vomit; place on side. Inhaled: get the patient out of the atmosphere only if it is safe for you; NRB 12–15 L/min. Injected (bites and stings): keep quiet, remove jewelry, immobilize, watch for anaphylaxis.
4. Snakebite: keep still and quiet; remove jewelry and tight clothing; keep the limb at or below heart level; immobilize; no tourniquet and no ice. Photograph the snake from a safe distance; never pick one up.
5. Radiation: most incidents are at facilities with experts on site. Without them, don't enter the area, keep bystanders out and keep victims there, unless a victim is seriously hurt; then treat life threats, remove them from the hazard, remove outer clothing and

wash exposed skin.

6. Emotionally disturbed patients: scene safety first; call law enforcement for anyone dangerous, armed or out of control; be direct and honest; avoid restraint, but once restraints are on, don't remove them.

Skills stations

- **A. Cooling and rewarming:** heat stroke cooling positions; hypothermia packaging.
- **B. Snakebite:** immobilize a lower leg bite with the limb below heart level.

Evaluation and common errors. Watch for confusing heat exhaustion with heat stroke, applying ice or a tourniquet to a snakebite, entering an unsafe atmosphere, and approaching a behavioral patient without law enforcement.

Instructor notes. Add the Poison Center number, 1-800-222-1222, to apparatus cards. Behavioral health calls can be hard on responders; mention the member assistance program.

Module E6 — Bleeding, shock, soft tissue and burns

Protocols: 2 Amputations, 3 Bleeding, 5 Burns, 6 Chest Injuries, 14 Evisceration, 23 Shock, Appendix 2 (Rule of Nines) · **Taskbook:** E.11–E.15 · **Time:** 3 hr class, 3 hr skills

Objectives. The trainee can control bleeding with and without an impaled object, recognize and treat hemorrhagic and cardiogenic shock, package an amputated part, dress an evisceration, manage rib fractures, a flail chest and a sucking chest wound, and classify and treat thermal, chemical and electrical burns using the Rule of Nines.

Materials. Bleeding-control manikin or wound packing trainer, commercial tourniquets and BP cuffs, trauma dressings, occlusive dressings, saline, plastic bags and a cooler, bulky pads and cravats, burn sheets, Rule of Nines chart, moulage, Module E6 slides.

Lecture outline

1. Bleeding without an impaled object (protocol 3): direct pressure and elevation; if bleeding persists, a BP cuff or tourniquet above the wound at the lowest pressure that stops it; bandage; treat for shock; vitals every 5 min.
2. Impaled object: pressure around it; stabilize it in place; remove an object in the cheek only if the airway can't be kept open; vitals every 10 min.
3. Shock (23). Hemorrhagic: control bleeding, NRB 10–12 L/min, splint fractures, elevate the legs, blankets under and over, vitals every 5 min. Cardiogenic: oxygen, semi-Fowler's (legs up only if blood pressure is low), blankets.

4. Amputation (2): control bleeding and treat shock; rinse the part briefly with saline (don't scrub), wrap in saline-moistened gauze, seal in a plastic bag, place in a cool container (not directly on ice), send it with the patient. Reattachment is possible up to 24 hours but best within a few.
5. Evisceration (14): moist sterile dressing, then an occlusive cover; never push organs back in.
6. Chest injuries (6): rib fractures, oxygen; flail chest, bulky pad and cravats (no tape), position on the injured side if tolerated; sucking chest wound, occlusive dressing taped on three sides, uncover it immediately if breathing gets worse.
7. Burns (5): stop the burning; remove jewelry and non-stuck clothing; dry sterile dressings; keep warm. Chemical: brush off dry lime first, then flush at least 20 minutes, no antidotes. Electrical: make sure the power is off; look for entry and exit wounds; expect cardiac and breathing problems, fractures and seizures.

Burn severity (protocol 5)

Severity	3rd degree	2nd degree	1st degree
Minor	Under 2% BSA (not face, hands, feet, groin or major joints)	Under 15%	Under 20%
Moderate	2–10% (not those areas)	15–30%	20–75%
Critical	Over 10%, or any 2nd or 3rd degree to face, hands, feet, groin or major joints, or with airway or other injuries	Over 30%	Over 75%

Rule of Nines (adult): head 9% (4.5% front, 4.5% back), each arm 9%, chest 9%, abdomen 9%, upper back 9%, lower back 9%, each leg 18% (9% front, 9% back), groin 1%. The patient's palm is about 1%. Report the burn degree, percentage and severity to the responding EMS unit.

Skills stations

- **A. Bleeding control:** direct pressure, then a tourniquet or BP cuff on the clock.
- **B. Amputated part:** package a moulage finger.
- **C. Chest and abdomen:** flail chest pad, three-sided occlusive dressing, evisceration dressing.
- **D. Burns:** estimate BSA on three moulage patients and classify severity.

Evaluation and common errors. Watch for slow escalation on major bleeding, putting an amputated part directly on ice, taping a flail segment, and cooling large burns until the patient is hypothermic.

Module E7 — Fractures, spine and multiple trauma

Protocols: 15 Fractures and Dislocations, 16 Head-Neck-Spine Injuries, 18 Multiple Trauma · **Taskbook:** E.16–E.17 · **Time:** 2 hr class, 3 hr skills

Objectives. The trainee can check pulse, movement and sensation below an injury, splint a fracture or dislocation as found (or with gentle traction when no pulse is present), hold manual spinal stabilization, open the airway with a modified jaw thrust, do a neuro exam before and after immobilization, immobilize to a long spine board, log-roll a vomiting patient, and work the multiple trauma priorities in order.

Materials. Rigid, SAM and vacuum splints, slings and swathes, cervical collars, long spine board with straps and head blocks, suction, BVM, Module E7 slides.

Lecture outline

1. Fractures (15): control bleeding; check pulse and sensation below the injury; if a pulse is present, splint as found; if absent, apply gentle traction while repositioning until the pulse returns, then splint (splint anyway if it doesn't return); vitals every 10 min; treat for shock if needed.
2. Splinting rules: immobilize the joint above and below a bone injury, and the bones above and below a joint injury; recheck pulse and sensation after splinting.
3. Head, neck and spine (16): contact EMS early; manual stabilization; no hyperextension, use the modified jaw thrust; if not breathing, BVM with oxygen per protocol; if conscious, NRB 12–15 L/min.
4. Bleeding from the ears or over a possible depressed skull fracture: don't apply direct pressure there.
5. Neuro exam before and after immobilization: level of consciousness, posture, sensation, movement, pupils.
6. Immobilize to a long spine board; vitals every 5 min; elevate the foot of the board for shock; if the patient vomits, suction and turn the patient on the board as a unit.
7. Multiple trauma (18), in order: modified jaw thrust; ventilate if not breathing; find and seal sucking chest wounds; control severe bleeding; hold the head until immobilized; brief exam; treat for shock.

8. Air evacuation: obvious spinal cord or closed head injury, and penetrating trauma to the head, neck or chest, are criteria (protocol V).

Skills stations

- **A. Splinting:** forearm, lower leg and shoulder dislocation as found, with pulse, movement and sensation checks.
- **B. Spinal care:** manual stabilization, collar, log roll and long board as a four-person team.
- **C. Trauma sequence:** rapid run-through of the multiple trauma steps on a moulaged volunteer.

Evaluation and common errors. Watch for letting go of the head too early, no neuro check before and after, forgetting to recheck distal pulses after splinting, and tilting the head to open the airway on a trauma patient.

Instructor notes. This is also where to discuss the spinal motion restriction question in the protocol notes, at instructor level only.

Module E8 — Childbirth and sensitive patients

Protocols: 8 Childbirth, 22 Sexual Assault, 25 Vaginal Bleeding, Appendix 3 (APGAR) · **Taskbook:** E.23–E.24 · **Time:** 2 hr class, 2 hr skills

Objectives. The trainee can take an obstetric history, recognize imminent delivery, assist a normal head-first delivery, clamp and cut the cord, keep the newborn warm, score APGAR at 1 and 5 minutes, deliver the placenta without pulling the cord, manage the listed complications, and care for vaginal bleeding and sexual assault patients with privacy and evidence in mind.

Materials. OB delivery manikin, OB kit (clamps, sterile scissors, bulb syringe, towels, infant blanket, basin or bag), pads, APGAR chart, Module E8 slides.

Lecture outline

1. History: age, last menstrual period, due date, multiple babies, gravidity and parity, abortions or miscarriages, contractions, bleeding or fluid, urge to move the bowels. Check the mother for seizures or fainting; vitals every 10 min.
2. Crowning means delivery is imminent: prepare to deliver on scene.
3. Normal delivery: gentle palm pressure on the head to prevent an explosive birth; suction mouth then nose with the bulb syringe; support the head; gentle downward guidance to deliver the top shoulder, then the bottom; be ready, the body comes fast.

4. Cord: keep the baby level with the vagina until clamped; first clamp 7–10 inches from the baby, second clamp 2–3 inches beyond it; cut between with sterile scissors.
5. Newborn: dry, suction as needed, place on the mother's abdomen wrapped up; turn off fans and AC; APGAR at 1 and 5 minutes.
6. Placenta: massage the uterus in circles; never pull the cord; bag the placenta for transport.
7. Complications. Membranes intact: tear them with fingers and clear the face. Cord around the neck: loosen and slip it over the head; if it won't loosen, clamp twice and cut between. Prolapsed cord: knees-to-chest or hips raised on pillows, NRB 12–15 L/min, wrap the cord in warm moist dressings, lift the baby's presenting part off the cord with a gloved hand and stay in place. Bleeding after delivery: treat for shock, pads at the opening (never pack), uterine massage.
8. Vaginal bleeding (25): pad beneath, vitals every 5 min, history (last period, contraception, sexual history), treat for shock, save any tissue passed.
9. Sexual assault (22): treat life threats; privacy; same-sex responder if possible; emotional support; keep the history medical; don't disturb the scene or remove clothing if possible; don't use the patient's name on the radio; no showering, toileting or changing clothes before transport; arrange clothing for the trip home.

APGAR (Appendix 3)

Sign	0	1	2
Activity (muscle tone)	Absent	Arms and legs flexed	Active movement
Pulse	Absent	Below 100	Over 100
Grimace (reflex)	Flaccid	Some flexion	Active motion (sneeze, cough, pull away)
Appearance (color)	Blue, pale	Body pink, extremities blue	Completely pink
Respiration	Absent	Slow, irregular	Vigorous cry

Total 7–10: excellent condition. 4–6: moderately depressed. 0–3: severely depressed.

Skills stations

- **A. Normal delivery** on the OB manikin, through placenta and APGAR.

- **B. Complications:** cord around the neck and prolapsed cord positioning.

Evaluation and common errors. Watch for pulling on the cord, letting the newborn get cold, and forgetting the second APGAR.

Instructor notes. Sexual assault and pregnancy-loss calls can be hard on crews and patients; keep the classroom discussion respectful and offer the member assistance program.

EMS scenarios 1–6: medical

Each scenario runs about 10 minutes plus a 5-minute debrief. The operator reads the dispatch, answers only what the team checks for, and gives vitals only when they are taken. Expected actions cite the protocol number; EMT-B and above steps are marked. Score each run on the evaluation sheet below.

Scenario 1 — Chest pain at the feed store

- **Dispatch:** 62-year-old man with chest pain, feed store on the highway.
- **On arrival:** Man sitting on a pallet, pale and sweating, rubbing his chest. He was loading feed 20 minutes ago.
- **Findings:** Pressure, 7/10, spreading to the left arm. History of high blood pressure; takes lisinopril; has a nitroglycerin prescription he has never used. No allergies. GCS 15.
- **Vitals:** BP 152/94, pulse 104, respirations 22, SpO2 95%, skin pale, cool and sweaty.
- **Expected actions (7):** scene safety and gloves; loosen clothing, position of comfort, keep him calm and still; NRB 12–15 L/min; OPQRST and SAMPLE; vitals every 3–5 min; AED staged nearby. EMT-B and above: 2 chewable 81 mg aspirin after checking for allergy; assist his nitroglycerin if systolic BP is 100 or higher, up to 3 doses, after checking the expiration date and asking about erectile dysfunction drugs.
- **Progression:** After the second nitroglycerin his BP is 96/60. The team stops nitroglycerin and moves to cardiogenic shock care (23): semi-Fowler's, legs up because he is hypotensive, blankets, vitals every 5 min.
- **Debrief:** Why every chest pain is treated as a heart attack; the systolic BP cutoff; what goes in the handoff.

Scenario 2 — Collapse at the fellowship hall

- **Dispatch:** 70-year-old woman collapsed, CPR in progress, church fellowship hall.
- **On arrival:** A bystander is doing compressions. A wall-mounted AED is unopened.

- **Findings:** Unresponsive, not breathing, no pulse.
- **Expected actions (10):** confirm arrest; take over CPR without a long pause; attach the AED; stop CPR and press Analyze; shock when advised with everyone clear; resume CPR right away; BVM with oxygen; rotate compressors every 2 minutes.
- **Progression:** First analysis: shock advised. After the second analysis: no shock advised, pulse present, breathing 6 per minute. The team checks the pulse, keeps ventilating with the BVM, and monitors ABCs.
- **Debrief:** Compression pauses, roles, the times and number of shocks for the handoff, and thanking and checking on the bystander.

Scenario 3 — Bee stings in the hay field

- **Dispatch:** 34-year-old man stung by bees, having trouble breathing.
- **On arrival:** Man sitting on a tailgate; hives on arms and neck, lips swelling, hoarse voice, wheezing. He has a prescribed epinephrine auto-injector in the truck.
- **Vitals:** BP 86/52, pulse 122, respirations 28 and wheezy, SpO2 89%.
- **Expected actions (1, 19, 23):** keep bees away from the crew; open and maintain airway; NRB 12–15 L/min, BVM if breathing fails; treat for shock; remove rings and watch (hands are swelling); keep him still; vitals every 3–5 min. EMT-B and above: confirm it is his auto-injector and not expired, then assist with it into the outer thigh.
- **Progression:** Five minutes after epinephrine: wheezing better, BP 104/70. If the team doesn't give or assist with epinephrine, his voice gets hoarser and SpO2 falls to 84%.
- **Debrief:** Signs that make it anaphylaxis; the five rights of medication assists; why he still goes to the hospital.

Scenario 4 — Confused at the kitchen table

- **Dispatch:** 55-year-old woman acting confused, history of diabetes.
- **On arrival:** Her husband says she took her insulin and skipped lunch. She is sweaty and slow to answer but follows commands and swallows on request.
- **Vitals:** BP 132/84, pulse 110, respirations 18, SpO2 97%, skin cool and sweaty. GCS 14 (confused).
- **Expected actions (12):** confirm she is alert enough, has a gag reflex and can drink unaided; orange juice with 2–3 tablespoons of sugar, or oral glucose (EMT-B and above); vitals every 5–10 min.

- **Progression (operator's choice):** She becomes unresponsive before drinking. The team stops anything by mouth, contacts EMS, suctions, places her in the recovery position and gives NRB 10–12 L/min.
- **Debrief:** When oral sugar is safe and when it is not.

Scenario 5 — Slurred speech at breakfast

- **Dispatch:** 78-year-old man, wife says he is talking funny.
- **On arrival:** Right facial droop, right arm drifts down, speech slurred. His wife says he was normal at 0700; it is now 0740.
- **Vitals:** BP 188/102, pulse 88 and irregular, respirations 18, SpO2 96%. GCS 14.
- **Expected actions (11):** airway, suction ready; NRB 12–15 L/min; neuro exam; semi-Fowler's; calm and reassure; vitals every 3–5 min; report the time he was last known well.
- **Progression:** He vomits and becomes harder to rouse. The team suctions, places him lateral recumbent, and is ready to ventilate with the BVM.
- **Debrief:** Why the last-known-well time matters for the hospital.

Scenario 6 — Hay crew collapse in July

- **Dispatch:** 19-year-old man passed out in a hay field, 101°F.
- **On arrival:** Crew moved him to the shade of a trailer. He is confused, skin hot and dry.
- **Vitals:** BP 98/60, pulse 136, respirations 28, SpO2 96%. GCS 13.
- **Expected actions (17):** contact EMS; move him to a cool place (an air-conditioned cab); remove heavy clothing; ice packs to head, neck, armpits and groin, or a wet sheet with airflow; stop cooling if he shivers; NRB 10–12 L/min; vitals every 5 min.
- **Progression:** He has a 1-minute seizure. The team protects him, puts nothing in his mouth, then positions and suctions (21).
- **Debrief:** Heat exhaustion versus heat stroke; why cooling starts before transport.

EMS scenarios 7–12: trauma, childbirth, refusal and burns

Scenario 7 — Motorcycle versus deer

- **Dispatch:** Motorcycle crash, one rider down, farm-to-market road at night.

- **On arrival:** Traffic is moving past. A 28-year-old man lies in the ditch, helmet on, talking. Bright red blood is spurting from a gash on his lower right leg. He says his neck hurts and his hands tingle.
- **Vitals:** BP 104/68, pulse 118, respirations 24, SpO2 94%. GCS 14.
- **Expected actions (3, 16, 18, 23):** block the lane and set up a protected work area (taskbook 1.16); manual spinal stabilization; airway by modified jaw thrust if needed; direct pressure, then a BP cuff or tourniquet above the wound when bleeding continues; treat for shock; neuro exam before and after immobilization; long spine board; vitals every 5 min.
- **Progression:** BP falls to 86/50 if bleeding control is slow.
- **Debrief:** Order of priorities in multiple trauma; whether tingling hands make this an air evacuation candidate (protocol V); traffic safety.

Scenario 8 — Chainsaw on the fence line

- **Dispatch:** Man cut his hand with a chainsaw, ranch off a county road.
- **On arrival:** 45-year-old man holding a towel on his left hand; his index and middle fingers are cut off at the middle knuckle and lying in the grass.
- **Vitals:** BP 128/80, pulse 108, respirations 20, SpO2 98%.
- **Expected actions (2, 3, 23):** shut down the saw; control bleeding; treat for shock; find both fingers; rinse briefly with saline, wrap in moist gauze, seal in a plastic bag, put the bag in a cool container (not directly on ice), and send them with him.
- **Progression:** The team asks EMS about air evacuation; amputations that may be reattached are on the criteria list.
- **Debrief:** Why the part is never put directly on ice; reattachment time window.

Scenario 9 — Baby won't wait

- **Dispatch:** 24-year-old woman in labor, EMS 25 minutes out.
- **On arrival:** Second pregnancy, 39 weeks, water broke 30 minutes ago, contractions 2 minutes apart, feels the urge to push. The baby's head is crowning.
- **Vitals (mother):** BP 124/78, pulse 96, respirations 20.
- **Expected actions (8):** history; open the OB kit; gentle pressure on the head; suction mouth then nose; deliver the shoulders; keep the baby level until clamped; clamps at 7–10 inches and 2–3 inches beyond, cut between; dry, warm and place on the mother; APGAR at 1 and 5 minutes; uterine massage; deliver the placenta without pulling the cord.

- **Progression (operator's choice):** The cord is around the baby's neck; the team slips it over the head. One-minute findings: active movement, pulse 140, cries when suctioned, body pink with blue hands and feet, vigorous cry (APGAR 9).
- **Debrief:** Keeping the newborn warm; when to reassess mother and baby.

Scenario 10 — Rattlesnake in the barn

- **Dispatch:** 9-year-old girl bitten by a snake.
- **On arrival:** The snake is under a pallet in the barn. Her mother carried her to the porch. Two puncture wounds on the left ankle; swelling spreading up the calf; she is crying and scared.
- **Vitals:** pulse 120, respirations 26, BP 100/64.
- **Expected actions (24):** stay away from the snake; keep her still and calm; remove her shoe, sock and ankle bracelet; keep the leg at or below heart level; immobilize; no tourniquet, no ice; vitals every 5 min; watch for anaphylaxis; a photo of the snake only if it can be taken safely.
- **Debrief:** Children's normal vitals; talking to a scared child and parent.

Scenario 11 — Rollover, "I'm fine"

- **Dispatch:** Single-vehicle rollover into a bar ditch, one occupant.
- **On arrival:** A 40-year-old man is out of the pickup and walking around. Small scrape on his forehead. He wants to go home.
- **Findings:** Alert and oriented to person, place, time and events. GCS 15. No alcohol odor. Vitals normal.
- **Expected actions (II, IV):** full assessment despite the refusal; explain the risks given the mechanism; confirm he is an adult who is conscious and fully oriented; complete the First Responder refusal form; cancel the responding unit per protocol IV; document.
- **Variant (operator's choice):** He asks the same question three times and can't say what happened. He is not fully oriented, so EMS is not cancelled; the team keeps him calm and asks law enforcement to stay.
- **Debrief:** What makes a refusal valid; protecting the patient and the department with good documentation.

Scenario 12 — Brush pile flash burn

- **Dispatch:** Man burned while burning brush.
- **On arrival:** A 67-year-old man used diesel on a brush pile; it flashed. The pile is still burning near a fence. His forearms and hands are red and blistered, his eyebrows and beard are singed, and his voice is hoarse.
- **Vitals:** BP 146/90, pulse 112, respirations 24, SpO2 95%.
- **Expected actions (5):** make the fire scene safe; stop the burning process; NRB 12–15 L/min; remove his watch and rings now; classify (2nd degree, both forearms and hands, about 9% BSA by the Rule of Nines, plus the face); dry sterile dressings; keep him warm; vitals every 3–5 min.
- **Severity:** Critical, because the hands and face are burned and the hoarse voice suggests an airway burn. Report degree, percentage and severity to EMS.
- **Debrief:** Airway burn warning signs; why level of care for major burns is an air evacuation criterion.

Scenario evaluation and handoff report

The team lead passes a scenario by meeting every critical item and at least 8 of the 10 performance items. Taskbook E.26 is signed after the trainee passes three scenarios as team lead: one medical, one trauma, and one of the trainee's choice.

Critical items (any miss fails the run)

- ☐ Scene safety checked and gloves on before patient contact
- ☐ Airway, breathing and circulation checked and life threats treated when found
- ☐ Severe bleeding controlled (trauma scenarios)
- ☐ No action outside the protocol or above the trainee's certification level
- ☐ Handoff report given to the arriving medic

Performance items

#	Item	Met
1	General impression and level of consciousness (GCS) stated	
2	SAMPLE history obtained	
3	OPQRST obtained when pain is present	

#	Item	Met
4	Physical exam (DCAP-BTLS) appropriate to the complaint	
5	Baseline vitals taken and repeated at the protocol's interval	
6	Protocol steps done in the right order	
7	Oxygen given at the protocol's rate and device	
8	Patient positioned per protocol	
9	Reassessed after each treatment and responded to changes	
10	Team directed clearly; patient and family kept informed	

Evaluator: _____ Date: _____ Pass / Retest

Handoff report format (30 seconds)

1. Age and sex, chief complaint.
2. What we found: mechanism or history, key exam findings, GCS.
3. Vital signs: first set and most recent.
4. What we did and when (oxygen, medication assists with times and doses, CPR and shocks, splints, tourniquet time).
5. How the patient responded, and anything still pending.

Example: "62-year-old man, chest pressure for 30 minutes after lifting feed, 7 out of 10, radiating to the left arm, history of high blood pressure. First BP 152 over 94, pulse 104, sats 95. Now 96 over 60 after two of his nitro at 1412 and 1417, so we stopped. Aspirin 162 milligrams at 1410, non-rebreather at 15 liters, legs up. He's alert and still at 6 out of 10."

Sources

- Wise County First Responder Protocols (general orders, sections IV–VI, protocols 1–25, Appendices 1–3), provided by the department. The copy used lacks Section III, Equipment List.

- GSVFD Firefighter Taskbook, EMS section E.1–E.26, and the GSVFD Firefighter I & II Instructor Guide.
- NFPA 1010 (2024), Chapter 10, Emergency Medical Services requirements set by the authority having jurisdiction.